

ROYAL HOSPITAL CHILD ABUSE CLINICAL PATHWAY

Evaluation Pathway for Children with Suspected Abuse

Up to 18YEARS

REDFLAGS – PHYSICAL MALTREATMENT:

- Any concern for injury in patient 4 months or below
- TEN-4-FACEs rule
- Non-ambulatory child with fracture or bruises
- Skull fracture or intracranial injuries in < 2yrs
- Any previous unrecognized healing fractures
- Any child with patterned bruising or unexplained/clearly demarcated burn
- Any child with symptoms/signs of trauma without an explanation
- Any child present during domestic violence incident.
- Injury was unwitnessed or incompatible with mechanism
- Injury or mechanism inconsistent with developmental stage of the child.
- Delay in seeking care >24 hours without satisfactory explanation.
- Single or multiple circular, deep-cratered burns consistent with cigarette burns.
- Multiple non-scald burns
- Contact burn in a location that is not consistent with a child touching a hot item unintentionally.

Follow the physical abuse clinical pathway

REDFLAGS – SEXUAL ABUSE:

Physical Indicators

- Difficulty in walking or sitting.
- Lack of bowel control.
- Bloody discharge in pre-menstrual girls or anal bleeding in boys.
- Venereal disease.
- Complaints of pain, itching or swelling in the genital area.

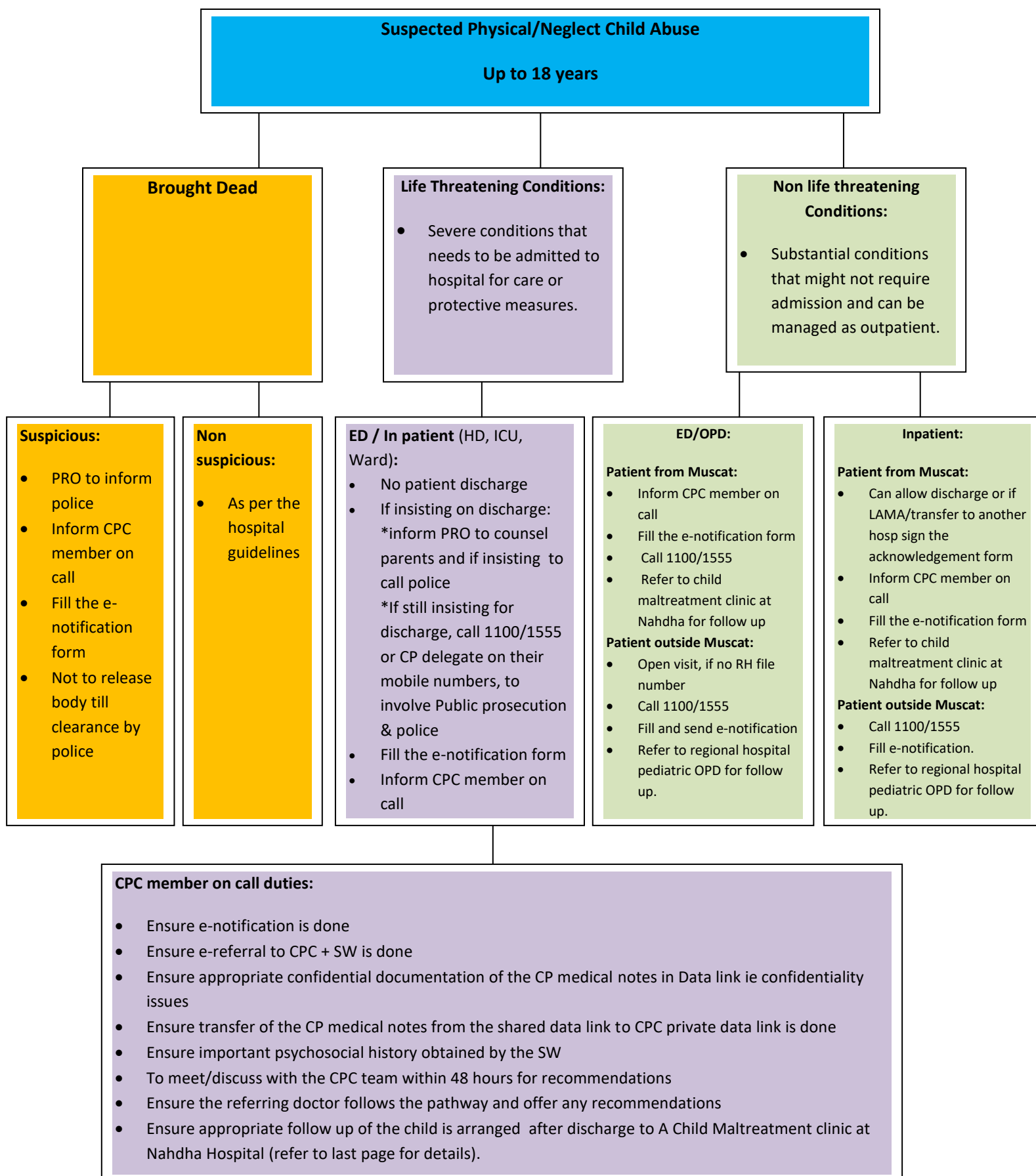
Indicators in Child's Behavior

- Child discloses he/she has been sexually abused.
- Bizarre or unusual sexual behaviors.
- Knowledge of sexual behaviors inappropriate for child's age.
- Excessive masturbation.
- Lack of self-esteem, depression.
- Child talks of "secrets" that cannot be told.

Parental Behaviors

- History is incompatible with child's presentation and physical signs
- Perceives the child as "different" or "bad".
- Belittles or humiliates the child.
- Insists that the child be treated differently.
- Expresses or shows no affection toward the child.

Follow the sexual abuse clinical pathway



Suspected Sexual Abuse

Up to 18 years

Non emergency:

- Reported to health facility after 72 hours of the event
- No active vaginal or rectal bleeding
- Presents with urogenital infections or behavioral changes.

Emergency

- Reported to health facility by (caregiver/child) within 72 hours of sexual abuse.
- Presented with vaginal /rectal bleeding with history suspicious of sexual activity.

- Does not require admission
- Open visit, if no RH file number

Patients from Muscat

- Inform CPC member on call
- Fill the e-notification form.
- Refer the patient to As Seeb polyclinic, child psychiatric 2 clinic for follow up (refer to last page for details)

Patient outside Muscat:

- Call 1100/1555.
- Fill the e-notification form.
- Refer to regional hospital pediatric OPD for follow up.

- Provide emergency care
- Call 1100/1555
- Inform CPC member on call
- Fill the e-notification form
- Admit the patient and arrange forensic medicine studies through PRO or CPC delegate (1100/1555)
NB: The forensic evaluation to be done at the ROP hospital within 72 hours.
(Forensic on call doctor's number: 91932297)
- Send sexual abuse workup as listed in the guideline.

CPC member on call:

- Ensure e-notification is done
- Ensure e-referral to CPC + SW is done
- Ensure appropriate confidential documentation of the CP medical notes in data link ie confidentiality issues
- Ensure transfer of the CP medical notes from the shared data link to CPC private data link is done
- Ensure important psychosocial history obtained by the SW
- Ensure forensic history is obtained
- Ensure forensic evaluation at the ROP hospital is done
- To meet/discuss with the CPC team within 48 hours for recommendations
- Ensure the referring doctor follows the pathway and offer any recommendations
- Ensure appropriate follow up of the child is arranged after discharge to Seeb Polyclinic

EVALUATION OF CHILD ABUSE BY TYPE

Presentation	Labs	Radiology/ Consults	Other Differential Diagnosis to consider
Suspected Abusive head trauma (AHT)/Shaken Baby Syndrome	CBC & Differentials Coagulation studies VWF profile Hemophilia A & B DIC panel (d-dimer and fibrinogen) Factor XIII	Skeletal survey if the child <2yrs, can consider for 24-36months Brain CT without contrast Brain +/- Spine MRI without a contrast if abnormal findings on CT	Accidental falls and injuries
			Bleeding disorders – hemophilia, liver disease, Vit K def
	Urine drug screen	Retinal examination for retinal hemorrhages in patients <12 months	Metabolic disease- Glutaricaciduria type I
			Connective tissue disease – Ehler Danlos syndrome
			Infection – Meningitis, encephalitis, sepsis
			Birth trauma
			Intracranial pathology – cyst, blood clots, ruptured aneurysm, tumors
Suspected Abusive Fracture	Calcium Phosphate Magnesium Alk phos 25-OH vit D PTH	Skeletal survey if the child <2yrs, can consider for 24-36months Brain CT if: -Age <12 months -Facial bruising -Altered mental status	Accidental injury
			Birth injury
			Pathological fracture ie OI, rickets, chronic disease and malignancy
Suspected Abusive Bruising	CBC with differential PT/PTT/INR VWF profile Factor VIII Factor IX Platelet function assay Factor XIII Troponin if bruising on chest wall	Skeletal survey if the child <2yrs, can consider for 24-36months Brain CT if: -Age <12 months -Facial bruising -Altered mental status.	Bleeding disorders
			Vitamin K deficiency
			Salicylate ingestion
			Immunoglobulin A vasculitis (Henoch-Schönlein purpura) and other vasculitides
			Dermal melanosis (formerly Mongolian spots)
			Hemangiomas
			Delayed subaponeurotic fluid mass

Suspected Abusive Abdominal Trauma	AST/ALT and lipase Consider Troponin is bruising on the chest wall	CT abdomen/pelvis with contrast if: -AST/ALT >80 -UA >20RBCs significant abdominal injury/bruising -Comatose patient	Accidental trauma
	Urine analysis (UA)	Skeletal survey if the child <2yrs, can consider for 24-36months Brain CT if: -Age <12 months -Facial bruising -Altered mental status	
Suspected Abusive Burns	CBC with differentials In cases of severe tissue injury: CPK, aldolase, myoglobin	Skeletal survey if the child <2yrs, can consider for 24-36months	Accidental burns
			Dermatological conditions: Impetigo Phytophotodermatitis Psoriasis Epidermolysis bullosa
			Skin reaction Chemical and irritant burns
			Complementary and alternative therapies
			Infections: Staphylococcus scalded skin syndrome Cellulitis
Unexplained fussiness or altered mental status in <12 months	CBC with differentials Coagulation studies	Skeletal survey	Accidental injuries
		Brain CT	Infections: Meningitis Encephalitis
		Retinal examination for retinal hemorrhages	Intracranial hemorrhage
			Metabolic changes
Suspected Sexual Abuse	Urine pregnancy test VDRL HIV Hepatitis Per-vaginal/anal swab Any recommended Forensic tests	Forensic history	Infections: -streptococcal vaginitis -candida infections -Varicella -Pinworms -Perianal cellulitis -Molluscum contagiosum. -Warts
		Forensic examination	Anal conditions associated with perianal bleeding , bruising -IBD
		Consult ID team for HIV	-Chronic constipation

		prophylaxis for all cases of suspected sexual penetration within 72hrs of the incident.	-Rectal prolapse
		Consult Obs & gynae for emergency contraceptive pills for post-pubertal girls suspected of sexual penetration within 72hrs of the incident.	Dermatological conditions in perianal / genital area -Contact dermatitis - Psoriasis, seborrhea
			Trauma / genital injuries
			Female genital mutilation

OPD follow up of all cases of abuse to be referred as follows:

Hospital	Nahdha hospital	Seeb Polyclinic
Clinic name	child maltreatment clinic	child psychiatric 2 clinic
Dr. Name	Dr. Waleed Al Ruzaiqi	Dr. Muna Al Shukeili
Type of Abuse	All General physical/Neglect abuse	All Sexual abuse or if physical abuse with associated psychiatric/complex problems

Appendix:

CPC: Child protection committee; for CPC oncall please refer to the Rota.

SW: Social Worker

PRO: Public relation officer

ED: Emergency department

OPD: outpatient department

OI: Osteogenesis imperfecta

TEN-4-FACEs: Bruising clinical decision rule: Torso, ears, neck, any bruise below 4 months of age, frenulum, angle of the jaw, Cheeks, eyelids, subconjunctivae, patterned bruising.

References:

1. Clinical guidelines on child abuse and neglect, MOH Oman, First Edition 2016.
2. Julia Saperia et al; When to suspect child maltreatment: summary of NICE guidance. BMJ. 2009.
3. NHSGGC child protection service: 'Pathways for children presenting to emergency department at the Royal hospital for children when there is a concern of Abuse or Neglect', 2019.

Written by:	Dr. Zuwena Al Harthy & Dr. Laila Al Shaqsy
Other contributors:	Child Protection committee members
Authorized by:	Dr. Khoula Al Said